

Treatment Plan

Confidential Clinical Document — DS-001

Client: Deepak S.	Age: 56
Presenting Issue: Sleep disturbance (poor deep sleep quality)	Suggestibility: 76% Physical
Date Prepared: 2026-10-02	Session 1 Date: 2026-10-01
Treatment Modality: Clinical Hypnotherapy (Zoom)	Planned Sessions: 6-8 (reassessed at Session 4)

Presenting Concerns

- Poor deep sleep quality despite adequate total hours (8+)
- High light sleep proportion (79%) with low deep/REM (21%)
- Waking unrefreshed, persistent daytime fatigue
- Difficulty falling asleep (~35 minutes onset)
- Frequent night-waking with difficulty returning to sleep

Clinical Assessment

Suggestibility Profile: 76% Physical / 24% Emotional. Direct, literal approach optimal. Engineering mindset responds to systematic, evidence-based framing.

Psychiatric Screen: No current medications for depression, anxiety, or psychosis. No suicidal ideation. No contraindications to hypnotherapy.

Prognosis: Favorable. High physical suggestibility correlates with strong somatic response. Client motivated, analytically engaged, committed to data tracking.

Treatment Protocol: 8-Session Sleep Restoration

SESSION	TITLE	FOCUS	KEY OBJECTIVE
1	Assessment + First Experience	ABS testing, Theory of Mind, first induction	Establish baseline, build rapport
2	The Installation	10 PHS anchored in deep trance with ideomotor checks	Core programming installed
3	Maintenance + Overactive Mind	Night-waking protocol, Processing Vault, Depth Anchor Chain	Address mid-sleep disruption
4	Independence Pivot	Self-hypnosis teach, Natural Pharmacy, autonomous ceremony	Client runs ceremony independently
5	Fine-Tuning	Adaptive paths based on 2 weeks of independent practice	Optimize remaining gaps
6	Graduation / Consolidation	Sleep Temple capstone, relapse prevention, turbo mode	Long-term independence
7-8	Contingency	Gap closure, recovery, or secondary issues	Only if clinically indicated

Progress Indicators

Progress is measured through client subjective experience and clinical observation:

MEASURE	BASELINE (SESSION 1)	TARGET BY GRADUATION
How you feel waking up (1-10)	4/10 (exhausted)	7-8/10 (refreshed)
Falling asleep experience	Difficult (~35 min)	Natural, effortless
Night-waking	Frequent, hard to return	Rare, returns easily

Daytime energy	Low, persistent fatigue	Consistent throughout day
Sleep ceremony compliance	Starting (0%)	Effortless nightly habit
Self-hypnosis confidence (1-10)	Not yet learned	8+/10 (fully independent)

Note on personal tracking: If you use a personal sleep tracker (watch, ring, phone app), that data is welcome as additional reference information for your own awareness. However, therapy progress and outcomes are assessed through how you feel and clinical observation — not device readings. Neither the therapist nor the practice has tested, validated, or endorsed any specific wearable device for clinical assessment.

Therapeutic Approach

- **Lane:** Physical suggestible (direct, literal, authoritative delivery)
- **Induction:** Arm-raise, progressive relaxation, eye fascination
- **Deepening:** Trophy staircase (20-step), reactional sets, heavy/light, arm rigidity
- **Suggestion style:** Post-hypnotic installation with ideomotor confirmation
- **Homework:** Nightly sleep ceremony (10:30 PM), 4-7-8 breathing, dream journal
- **Decision points:** Session 4 (independence readiness), Session 5 (graduation readiness)

Roles and Responsibilities

Therapist will:

- Conduct sessions professionally via Zoom (60 min each)
- Provide After Visit Summary after each session
- Customize approach based on client feedback and clinical observation
- Maintain session notes per HIPAA requirements
- Provide Sleep Protocol Handbook after Session 2
- Offer mid-week check-ins as clinically appropriate

Client will:

- Attend all scheduled sessions (reschedule with 24-hour notice)
- Practice the nightly sleep ceremony consistently (10:30 PM)
- Report on sleep experience at each session (how you feel, notable changes)
- Maintain a dream journal
- Communicate medication changes, new symptoms, or concerns promptly
- Refrain from using alcohol as a sleep aid

Evidence Base

This protocol is informed by peer-reviewed research on hypnotherapy for sleep:

- Cordi MJ et al. (2014). "Deepening sleep by hypnotic suggestion." *Sleep*, 37(6), 1143-1152.
- Chamine I et al. (2018). "Hypnosis and upper airway involvement in obstructive sleep apnea." *Sleep Medicine Reviews*, 37, 45-58.
- Lam TH et al. (2015). "Hypnotherapy for insomnia: A systematic review." *Complementary Therapies in Medicine*, 23(5), 719-732.

AI Disclosure

This practice uses AI tools (Claude by Anthropic) for clinical documentation, session script preparation, and administrative support. All clinical decisions are made by the therapist. AI-generated content is reviewed before use. No client PHI is shared with AI systems without appropriate safeguards.

SB 577 Disclosure: Jithendran Sellamuthu is not a licensed physician, psychologist, or psychiatrist. Services are provided for vocational/avocational self-improvement under CA B&P Code §2908. Hypnotherapy is not a substitute for medical or psychological treatment.

No Guarantee: VRishi Hypnotherapy does not guarantee specific outcomes, cure rates, or timelines. Progress indicators are research-informed targets, not promises. Individual results vary based on suggestibility, compliance, health history, and other factors.

Therapeutic Exclusions: Clients on anti-depressant, anti-psychotic, or anti-anxiety medications, or with psychiatric history/suicidal ideation, must be referred to licensed mental health professionals.

Liability: Professional liability coverage maintained. Details available upon request.

Confidentiality: All client information is kept confidential per HIPAA Privacy Rule (45 CFR §§160, 164) and California law, except where disclosure is required by law. Records retained for 7 years per HIPAA §164.530(j).

Client: Deepak S.

Therapist: Jithendran Sellamuthu, C.MH.

Signature / Date

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